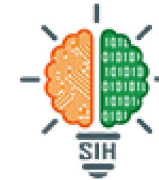


PATIENT CASE-TAKING SOFTWARE



SMART INDIA
HACKATHON
2025



Problem Statement

The Current Challenges

- Doctors spend valuable time on repetitive history-taking.
- Patients often forget or don't have past medical records.
- No unified system to maintain longitudinal health history across different doctors.
- Lack of early detection of disease outbreaks.
- Need for accessible, low-bandwidth and emergency access solutions.



Our Solution

AI-assisted patient intake + longitudinal digital health history

- Patient completes a dynamic, MCQ-first intake before consultation — at home or while waiting.
- AI adapts the next question from previous answers; captures complaint, duration, severity, red flags, medications, allergies and relevant history.
- Generates a structured doctor-ready summary + flags missing critical information.
- One longitudinal record stores cases, diagnoses, prescriptions, uploaded documents and vitals.



Innovation / Uniqueness

A complete, end-to-end workflow

- ✓ Branching AI questionnaire (adaptive MCQs)
- ✓ Missing-information detection (critical red flags)
- ✓ OCR for prescription and document digitization
- ✓ Low-bandwidth fallback (text-only + SMS)
- ✓ Longitudinal health history + multi-doctor access
- ✓ Population-level outbreak surveillance
- ✓ Combines modern (allopathy) and AYUSH intake in one platform



Core Modes

Flexible for different healthcare needs



General Mode

Standard medical case taking for all patients



AYUSH Mode

Captures Prakriti, Vikriti, Ahaar, lifestyle and other Ayurveda-specific details



History Management

Organised timeline of past cases, diagnoses, prescriptions, reports



Outbreak Detection

Analyzes de-identified data for regional symptom trends



Emergency & Low Bandwidth Access

SMS Based access to crucial data + text-only website mode



How It Addresses the Problem

Real solutions to real challenges

- ✓ Cuts repetitive history-taking → more doctor time for examination & decisions.
- ✓ Reduces paper/document hunting and improves continuity when patients change doctors.
- ✓ Aggregated, de-identified symptom trends can support early outbreak signals.

